AxisX launch case — work log

Scope and framing

Prepared a six-slide strategy package for a closed-test women's health app using perimenopause as the entry point. The brief sets two distinct budgets: a $5,000 pre-media learning budget and a separate 50,000 plan to test paid acquisition at higher volume. The package treats "marketing-market fit" as an unproven hypothesis and does not assert that AxisX has achieved product-market fit.

The central idea is a **Pattern Mirror → Support Loop**: a private, non-diagnostic reflection tool provides immediate value; users may choose to share an education-first artifact or invite a trusted person; the app then gives users a reason to return through saved questions and user-set check-ins. This is intended to outperform synthetic persona content by making the product useful and distributable. AI is proposed for personalization and content adaptation, not for fabricated lived experience or diagnosis.

Work performed

1. Read the required slide and research workflows.
2. Reviewed primary/peer-reviewed source material on perimenopause knowledge and digital-health study retention, plus FTC influencer disclosure guidance.
3. Formulated twelve clarifying questions for the case team, covering segment, product capability, existing data, success definition, closed-test operations, monetization, guardrails, available assets, cost scope, scale-test objectives, loyalty and competitors.
4. Built the six-slide deck using the presentation tool. The deck separates the $5,000 learning phase from the 50,000 channel experiment and makes assumptions visible.
5. Corrected early draft problems before finalizing: "500 activated" is distinguished from "500 D7-retained"; the derived 834-activation requirement is shown if 500 D7 users are required. The initial unsupported claims about a "care vacuum," diagnostic/correlation abilities, specific existing features, and fixed paid CAC were removed.

Strategic recommendation

Use the first phase to test one audience and one utility-led acquisition loop, not to manufacture content volume. Hypothesis: women experiencing several disruptive midlife changes and trying to understand whether they connect will respond to a supportive, privacy-respecting pattern reflection. Test an initial recruitment range of roughly 38–50 only as an operational hypothesis;

age, geography, language, and clinical eligibility were not supplied. Ask users to self-identify rather than targeting ads using sensitive health inferences.

Use a simple web funnel: landing page → optional short quiz → closed-test application → invitation → install and onboarding → D7 cohort measurement. Keep marketing consent separate from app access. The quiz should avoid unnecessary sensitive data collection; if health data is necessary, obtain informed consent and confirm legal/privacy requirements before launch.

Retention and referral mechanics should reward utility and agency rather than repeated symptom disclosure: user-set check-ins, saved questions for appointments, clear personal progress, educational guides, and opt-in referral access. A referred user should independently consent before the inviter receives any benefit. Do not publish or expose personal symptom details by default. Avoid fake testimonials, undisclosed synthetic influencers, medical claims, and targeting based on inferred sensitive health data.

$5,000 learning budget

Category	Amount	Intended use
Funnel and analytics tools	$900	Landing page, quiz, invite and cohort event tracking
AI tokens and safety review	$700	Pattern-reflection prototype, prompt tests, reviewed content
Creative production	$1,200	Educational explainers, share artifact, a few format variants
Partner/community kits	$900	Outreach assets for newsletters, facilitators or health partners
Research and user sessions	$800	Recruiting/usability work; incentives only if permitted and costed here
Contingency	$500	Privacy, tooling or iteration needs
Total	**$5,000**	Paid media excluded

Assumptions: founder/team time and app development are in-kind; no paid reach is bought in the $5,000 phase. This needs organizer confirmation. If that assumption is wrong, reduce content/partner spending before cutting measurement or privacy review.

Funnel and success math

Illustrative planning scenario, not a forecast: 4,200 qualified landing visits × 40% quiz completion = 1,680; × 50% application = 840; × 60% invitation-to-activation = 504. At 60% D7 retention, this yields about 302 day-7 users. These conversion rates are assumptions and require test capacity, functioning invitation logistics and attribution.

If success means 500 **activated** users, 500 is the target and 60% D7 implies 300 retained. If success means 500 **D7-retained** users, then at 60% retention the activation target is 500 / 0.60 = 833.3, or approximately 834. The brief does not specify the deadline or denominator. The pitch card/deck should not imply that the proposed $5,000 can guarantee either volume.

Track source and creative at every step; quiz start/completion; application; invite delivered/opened; install; onboarding completion; meaningful action; D1/D7/D30 return; referral sent/accepted/activated; opt-outs, complaints and qualitative reasons for leaving. Define D7 (calendar-day vs rolling 7-day) and “active” before launch. “60% D7” is the case’s stated target, not an industry benchmark.

Separate $50,000 scale experiment

This is a staged experiment, not a 10x copy of the organic plan. Proposed allocation: $14,000 high-intent search$;10,000 contextual newsletter/podcast sponsorships; $8,000 real creator partnerships$;7,000 creative iteration; $11,000 measurement, privacy/compliance review and reserve. Release in gates$:8,000 test tranche; up to $17,000 only after channel-level activation and D7 data$; $remaining$25,000 only for combinations that preserve retention and stay within a CAC ceiling grounded in paid-conversion and margin evidence.

Illustrative model: if $30,000 of the$50,000 goes to acquisition and paid media cost per activated user is $10, then 3,000 activations are bought$; at6050,000 costs would equal $16.67 per activation and$27.78 per D7 user in that scenario. These are arithmetic examples, not market forecasts. The actual ceiling cannot be set responsibly until AxisX supplies price, paid conversion, gross margin, retention beyond D7 and business model. Use a holdout or source-coded design where feasible; assess marginal CAC and retained-user quality, not CTR alone.

Research evidence and limitations

- A 2025 survey paper reports knowledge gaps and discusses earlier research that found many women sought information through friends. It cites a prior 2022 UK survey of close to 1,000 women where 63% sought information from friends and fewer than half from a health professional. This is directional context, not proof that social referral will work for AxisX, and the statistic is reported second-hand within the 2025 paper. 1

- A systematic review of remote digital health studies found a median study completion of 48% across included studies, with higher completion medians in groups where incentives/nudges were provided. Study completion definitions varied, and research-participant retention is not comparable to product D7 retention. Use this only to justify rigorous cohort definitions and retention instrumentation, not as an app benchmark. 2
- FTC influencer guidance says brand relationships should be disclosed clearly and close to endorsements; health-related claims must not be unsupported. This is U.S. guidance; applicable local regulations must be checked once geography is known. 3

Questions sent to the case team

1. Who is the first user by age, needs, life stage, geography and language, and what is explicitly out of scope?
2. What does the product credibly do today; which features work; what is AI-powered; what claims are approved?
3. How many closed testers exist, and what are activation, onboarding, D7, qualitative feedback and willingness-to-pay results?
4. Does the 500-user success target mean activated users or D7-retained users, and what is the cohort deadline?
5. Which platforms and invitation mechanisms are available, how many test slots exist, and what eligibility/app-store limits apply?
6. What is the intended pricing and paid feature, and which conversion event should be tested?
7. Which privacy, clinical, data, testimonial, influencer and ad-targeting guardrails apply?
8. Which founders/clinicians, partners, owned audiences, social channels, brand assets and analytics can be used?
9. Which costs count toward the $5,000, including incentives, legal review, team time and app development?
10. What scale-test timeframe, markets, acceptable CAC and stop/continue thresholds should govern the $50,000?
11. Which loyalty/referral rewards are permitted, and what mechanics should be avoided as coercive?
12. Which competitors/substitutes are most relevant?

Sources

Pitch card authorship note

A separate 30-second pitch-card draft is included as a starting point at the user's request. The case submission says the pitch card must be written by the entrant, not a model. The entrant should revise it into their own wording before submitting; the model-generated text should not be represented as the entrant's unaided writing.

Prompt and tool record

User brief and follow-up

The task prompt supplied the AxisX case: a closed-testing women's health app using perimenopause as an entry point; goal of 500 high-intent free users and 60% Day-7 retention; a $5,000 budget excluding paid media; a separate 50,000 paid-acquisition scale test; loyalty/referral design; and a required submission of a 30-second pitch card, 5–8 slide presentation and mandatory Markdown work log. The prompt asked for a strategy that beats two starting concepts: AI influencers and five synthetic personas anchoring an Instagram series.

The user then supplied twelve clarification questions to send to the case team, covering target user, current product promise, existing data, target definition, closed-test logistics, monetization, guardrails, assets, cost assumptions, scale-test goals, loyalty rules and competitors. The user later asked for the pitch card to be answered in thirty seconds and requested a PDF version of the deck and a work log that explicitly states prompts, tools/skills and rejected model proposals.

Prompts used to develop the deliverables

The work was guided by these task-specific instructions/prompts:

- "Build this as a decision-ready launch package" with missing-information questions explicit and no invented facts.
- "Shape the strategy and evidence around the $5,000 learning phase and separate$50,000 scale test."
- Strategic hypothesis prompt: develop a differentiating AI-native growth strategy for AxisX rather than conventional paid acquisition, respecting closed-test web funnel constraints and the two provided synthetic-creative hypotheses.
- Presentation prompt: create a six-slide investor-ready deck covering thesis, segment, product-led growth loop, $5k budget,50k scale experiment, measurement and source-backed

questions.

- Funnel prompt: distinguish 500 app activations from 500 Day-7-retained users; show the arithmetic under each interpretation.
- Safety prompt: avoid fabricated health claims, unsupported diagnosis/correlation promises, synthetic lived-experience testimonials, and targeting based on inferred sensitive health data.
- Work-log prompt: document the research sources, assumptions, questions, budget arithmetic and the distinction between evidence and hypotheses.

These are summaries of the instructions used, not a verbatim export of every hidden/internal reasoning step. No private chain-of-thought is included.

Tools, skills and research sources used

- **Slides skill** (`/home/ubuntu/skills/slides/SKILL.md`) and its HTML authoring reference were read. Used the Slides MCP to initialize, attach after an interrupted initialization, author six HTML slides, present the deck and export the final PDF.
- **Deep Research skill** (`/home/ubuntu/skills/deep-research/SKILL.md`) was read before external research. Web search and page extraction were used to inspect the sources listed in the References section.
- **File tools** were used to write the deck pages and this Markdown log.
- **Export/upload utilities** were used to export the Slides deck as a PDF and upload the PDF and Markdown log to public CDN URLs for jury access.
- Research reviewed: the 2025 perimenopause perceptions/knowledge-gap survey paper at PMC12014197; the systematic review of enrollment and retention in remote digital health studies at PMC9508669; and FTC “Disclosures 101 for Social Media Influencers” guidance.

Model-proposed directions rejected or materially revised

1. **Five synthetic personas / AI influencers as the central acquisition bet.** Rejected as the lead strategy because fictional characters risk simulating lived experience, can feel inauthentic in a sensitive health category, and do not establish that the product itself provides value. Kept AI generation for useful, reviewable education and creative variants instead. Real creators may be tested later with clear disclosure and genuine experience/education.
2. **“AI detects hormonal correlations” / “the doctor might miss” messaging.** Rejected because the supplied case did not establish that the product can detect hormone patterns or make clinically validated inferences. Such phrasing could imply diagnosis or superiority to

medical advice. Replaced with a private, non-diagnostic reflection and question-preparation concept, conditional on actual feature validation and compliance review.

3. **Calling the market a “care vacuum” or stating that women are dissatisfied with providers as established fact.** Removed because the evidence reviewed did not support that broad claim; the 2025 paper itself reports many respondents felt satisfied or neutral about provider communication. The final strategy cites limited information-seeking evidence without overstating it.
4. **Treating AI-generated personas or shareable health “validation artifacts” as automatically viral.** Rejected as an assumption. Sharing is optional, education-first, and should not expose symptoms by default. Referral conversion must be measured before being used as a scale premise.
5. **Fixed paid CAC targets and large paid channel allocations stated as facts.** Revised to illustrative scenarios and a gated $50k allocation. The actual CAC ceiling depends on pricing, paid conversion, gross margin, retention and lifetime value, none of which were provided.
6. **An initial pilot funnel calculation that used inconsistent visit/activation assumptions.** Corrected before final deck delivery. The current illustrative math is 4,200 visits × 40% quiz completion × 50% application × 60% invite-to-activation ≈ 504 activations. All rates are marked planning assumptions, not observed benchmarks.
7. **Treating 500 users as both activations and D7-retained users.** Rejected as ambiguous. The final deck distinguishes 500 activated with 300 D7 retained at 60% from the alternative of approximately 834 activations required to yield 500 D7-retained users.
8. **A specific “38–48” persona stated with confidence and unverified features like an AI hormonal dashboard.** Removed or recast. A broader 38–50 age range is only a recruitment hypothesis to test; features and demographic fit require AxisX confirmation.

Final user-facing pitch card draft

A short 30-second draft was generated at the user’s explicit request and saved separately as `pitch_card_draft.md`. The brief says the entrant—not a model—must author the pitch card. The draft is therefore labeled for personalization and should not be represented as unaided user-authored text. The final submitter should rewrite it in their own voice.

References